

Psychiatric Medication Management Follow-Up

SOAP Note Template for Psychiatric Nurse Practitioners · De-identified patient fields · Fillable PDF

SESSION INFORMATION

Date of session 2026-10-06 Duration (min) 30 Visit type Follow-up, telehealth

Patient ID (de-identified / MRN hash / initials code — no demographics)

SAMPLE-001

Primary reason for visit / Chief complaint

Follow-up for anxiety and sleep

S — SUBJECTIVE

Medication adherence, perceived benefits & side effects

- | | | | |
|--|--|---|---|
| ☒ Adherent as prescribed | ☐ Partial / inconsistent | ☐ Missed doses reported | ☐ Stopped / changed without advice |
| ☐ Perceived benefit: Yes | ☒ Benefit: Partial | ☐ Benefit: None / unclear | ☐ No side effects reported |
| ☒ Mild side effects | ☐ Moderate / bothersome SE | ☐ Severe SE / intolerability | ☐ Questions about regimen |

Additional details / narrative

Takes sertraline 50 mg daily. Mild nausea for the first hour after the dose. No missed doses this interval.

Depression symptoms since last visit

- | | | | |
|---|------------------------------------|--|---|
| ☐ None / remitted | ☐ Mild | ☐ Moderate | ☐ Severe |
| ☐ Low mood | ☐ Anhedonia | ☐ Guilt / worthlessness | ☐ Passive SI thoughts* |
| ☐ Improved vs last visit | ☐ Unchanged | ☐ Worsened | ☐ PHQ discussed/reviewed |

Additional details / narrative

Anxiety symptoms since last visit

- | | | | |
|---|---|--|---|
| ☐ None / remitted | ☐ Mild | ☒ Moderate | ☐ Severe |
| ☒ Excessive worry | ☐ Panic symptoms | ☐ Avoidance | ☐ Somatic anxiety |
| ☐ Improved vs last visit | ☒ Unchanged | ☐ Worsened | ☐ GAD scale reviewed |

Additional details / narrative

Ongoing worry about work deadlines. No panic attacks since last visit.

Sleep

- | | | | |
|---|--|---|--|
| ☐ Adequate / restorative | ☒ Onset difficulty | ☐ Maintenance issues | ☐ Early awakening |
| ☐ Hypersomnia | ☐ Nightmares | ☐ Apnea concern | ☐ Sleep aids used |

Additional details / narrative

Takes 45 to 60 minutes to fall asleep. No nightmares.

Psychiatric Medication Management Follow-Up (SOAP) — continued

Appetite / weight concerns

- | | | | |
|--|---|---|--|
| ☐ Stable appetite | ☐ Decreased | ☐ Increased | ☐ Weight loss concern |
| ☐ Weight gain concern | ☐ Restrictive patterns | ☐ Binge patterns | ☐ Med-related change |

Additional details / narrative

Attention / concentration

- | | | | |
|--|---|--|--|
| ☐ Intact / baseline | ☐ Mild difficulty | ☐ Moderate difficulty | ☐ Severe difficulty |
| ☐ Distractibility | ☐ Task completion issues | ☐ Forgetfulness | ☐ Improved on treatment |

Additional details / narrative

Energy levels

- | | | | |
|--|---|---|--|
| ☐ Normal / adequate | ☐ Low energy / fatigue | ☐ Marked fatigue | ☐ Elevated / restless |
| ☐ Diurnal variation | ☐ Improved | ☐ Unchanged | ☐ Worsened |

Additional details / narrative

Substance use since last appointment / current use

- | | | | |
|--|--|---|---|
| ☐ None reported | ☐ Alcohol | ☐ Cannabis | ☐ Nicotine / tobacco |
| ☐ Stimulants (non-Rx) | ☐ Opioids | ☐ Benzodiazepines (non-Rx) | ☐ Other substances |
| ☐ Increased vs last visit | ☐ Decreased / abstinent | ☐ Unchanged use | ☐ Clinical concern |

Additional details / narrative

Current functional impact & psychosocial context

- | | | | |
|--|---|--|--|
| ☐ Work/school stable | ☐ Work/school impaired | ☐ Home functioning OK | ☐ Home functioning impaired |
| ☐ Social functioning OK | ☐ Social withdrawal / conflict | ☐ Active stressors | ☐ Supports present |
| ☐ Housing concern | ☐ Legal / financial stress | ☐ Caregiving demands | ☐ Function improved |

Additional details / narrative

Safety risk assessment (document clinically important negatives)

- | | | | |
|--|---|--|--|
| ☒ No suicidal ideation | ☐ Passive SI | ☐ Active SI | ☐ Plan present |
| ☐ Intent present | ☐ Means access concern | ☒ No homicidal ideation | ☐ Homicidal ideation |
| ☐ Non-suicidal self-harm | ☒ No self-harm | ☒ Protective factors present | ☐ Safety plan reviewed |
| ☐ Crisis resources provided | ☐ ED / higher care discussed | ☐ Collateral obtained | ☐ Acute risk — escalate |

Additional details / narrative

Denies suicidal ideation, homicidal ideation, and self-harm. Identifies partner as a support.

Psychiatric Medication Management Follow-Up (SOAP) — continued

Other / custom complaints (not covered by categories above)

Use custom prompts for any symptoms or concerns that do not fit the listed subjective criteria.

Custom prompt 1

Custom prompt 2

Custom prompt 3

Additional notes / details for custom or unlisted complaints

O — OBJECTIVE

Vitals / monitored metrics (as obtained this visit)

BP HR Weight Other

Mental status exam (brief)

- | | | | | |
|--|---|---|---|--|
| ☐ Appearance WNL | ☒ Cooperative | ☒ Speech WNL | ☐ Mood/affect congruent | ☒ Thought process linear |
| ☒ No psychosis evident | ☒ Oriented x3 | ☐ Insight/judgment fair+ | ☐ Abnormal findings → note | ☐ MSE not fully documented |

Objective narrative / abnormal findings

Alert and cooperative. Speech normal rate. Thought process linear. No psychotic symptoms observed.

A — ASSESSMENT

Working diagnoses / clinical impression (ICD codes optional)

Generalized anxiety disorder

Response to treatment / clinical status vs last visit

- | | | |
|-----------------------------------|--|--|
| ☐ Improved | ☒ Partial response | ☐ Stable / maintained |
| ☐ Worsened | ☐ New / recurrent symptoms | ☐ SE impacting plan |

Partial response to sertraline 50 mg. Anxiety remains moderate and sleep onset is still delayed.

P — PLAN

Monitoring & medication reconciliation (check + short note)

- | | |
|---|--|
| ☒ Medication reconciliation | Continue sertraline 50 mg daily. Reassess dose at next visit if sleep and worry are unchanged. |
| ☐ PMP / PDMP review | |
| ☐ Blood pressure monitoring | |
| ☐ Heart rate / cardiac monitoring | |
| ☐ Weight / metabolic monitoring | |
| ☐ Labs ordered / reviewed | |
| ☐ Other health metrics | |

Informed consent for medication (discussed / documented)

- | | |
|--|--|
| ☒ Indication & expected benefits | |
| ☒ Common side effects discussed | |
| ☐ Serious risks discussed | |
| ☐ Time to effect / adequate trial | |

Psychiatric Medication Management Follow-Up (SOAP) — continued

- ☐ Adherence & not stopping abruptly
- ☐ Alternatives / risks of no treatment
- ☐ Patient questions addressed
- ☒ Patient agrees to plan

Consent / medication plan narrative

Brief psychotherapy / skills interventions this visit (check + note)

- ☐ Supportive therapy
- ☐ CBT techniques
- ☐ DBT skills
- ☐ ACT approaches
- ☐ Mindfulness
- ☐ Coaching / motivational
- ☐ Exec. functioning skills

Therapy / interventions narrative & patient response

Follow-up & disposition

- | | | |
|---|---|--|
| ☒ Routine follow-up | ☐ Earlier follow-up | ☐ Referral placed |
| ☐ Crisis plan in place | ☐ Return precautions given | ☒ Telehealth OK next |

Next appointment / disposition notes

Follow up in 4 weeks by telehealth. Return sooner for worsening anxiety or any safety concern.

Clinician (name/credentials)

Sample Clinician, PMHNP

Date signed

2026-10-06

After completing the form — SOAP note generation instruction

Generate a concise psychiatric medication-management SOAP note using only the information entered in this form. Do not invent findings. Preserve clinically important negatives, explicitly document safety findings, and separate Subjective, Objective, Assessment, and Plan. When information is not provided, omit it or state that it was not documented rather than inferring it.